



Invited Commentary | Equity, Diversity, and Inclusion

Breast Cancer Incidence in Asian American, Native Hawaiian, and Pacific Islander Populations

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Gomez et al¹ report a consequential and overdue finding: Breast cancer incidence is rising across nearly all examined Asian American, Native Hawaiian, and Pacific Islander populations in the US. They found especially concerning increases among women younger than 50 years; in distant-stage disease; and for more aggressive subtypes, including triple-negative, hormone receptor-negative and *ERBB2* (formerly *HER2/neu*)-positive, and hormone receptor-positive and *ERBB2*-negative tumors. Using updated National Cancer Institute Surveillance, Epidemiology, and End Results data from 14 states and disaggregating 9 Asian American, Native Hawaiian, and Pacific Islander ethnic populations, the authors reveal substantial heterogeneity in both incidence levels and trends that would remain largely obscured if Asian American, Native Hawaiian, and Pacific Islander populations were examined in aggregate. Their study is, therefore, more than an update on breast cancer incidence; it is a compelling demonstration that data equity—the principle that all individuals should benefit from data science—is foundational to cancer surveillance, etiologic research, and an effective public health response.²⁻⁴

For too long, Asian American, Native Hawaiian, and Pacific Islander populations have been aggregated in ways that flatten important differences in cancer burden.²⁻⁴ In reality, this population is exceptionally diverse, encompassing US-born individuals and immigrants alike who trace their heritage to dozens of nations, including Indigenous Pacific Islander peoples. Each subgroup carries distinct migration histories, sociocultural contexts, and socioeconomic profiles that shape their unique health care access, language needs, and risk factors.²⁻⁴ In Gomez et al,¹ this heterogeneity is not subtle. Incidence rates varied more than 3-fold across groups, with particularly high burden among Native Hawaiian women and rapid recent increases among multiple Asian American populations, including Asian Indian and Pakistani, Chinese, Korean, and Vietnamese women. Particularly concerning were the rising trends among women younger than 50 years and the increasing incidence of distant-stage disease, patterns that underscore the urgency of these findings for both public health and clinical care.^{1,5}

The importance of disaggregated data extends well beyond a more precise description. Aggregation can mask risk in communities that may otherwise be presumed to have low cancer burden, undercut the visibility of emerging trends, and delay the design of effective interventions.²⁻⁴ Incidence patterns, stage at diagnosis, and subtype distributions are especially relevant in breast cancer and can inform both etiologic hypotheses and prevention strategies.^{1,6} For clinicians, the findings from Gomez et al¹ argue against a one-size-fits-all understanding of breast cancer risk in Asian American, Native Hawaiian, and Pacific Islander populations. For policymakers and researchers, they reinforce that the categories used in data systems can either illuminate inequities or conceal them.²⁻⁴ More broadly, commonly used aggregated descriptions of breast cancer risk in Asian American, Native Hawaiian, and Pacific Islander populations, including in medical education and reference materials, may inadvertently perpetuate the misconception that these populations are uniformly at lower risk, when in fact, subgroup-specific incidence patterns are far more heterogeneous.³

At the same time, these trends should not be interpreted simplistically as evidence of an innate biologic difference across racial or ethnic groups. Race and ethnicity are social and historical constructs, and caution is warranted in how racial and ethnic differences in cancer incidence are interpreted.³ Such patterns likely reflect the complex interplay of migration, acculturation, structural

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conditions, reproductive and lifestyle transitions, screening patterns, and access to care.^{1,5,6} These structural conditions likely differ across Asian American, Native Hawaiian, and Pacific Islander populations and may include variation in insurance coverage, language access, affordability of care, immigration-related barriers, and other social determinants that influence timely screening, diagnostic follow-up, and treatment access. Gomez et al¹ appropriately note that the rapid incidence increases observed in some Asian American, Native Hawaiian, and Pacific Islander populations may mirror patterns seen in parts of Asia and may be associated with changing exposures across birth and immigration cohorts. The increases in advanced-stage disease also suggest that these patterns are not explained solely by greater screening.¹ Instead, they may point to a more complex reality in which changing risk exposures and inequities in awareness, screening access, timely detection, and follow-up are occurring simultaneously.^{1,7}

The findings are especially important in the context of broader concern about early-onset cancers in the US.⁵ That nearly all examined Asian American populations showed increasing early-onset breast cancer trends should prompt greater attention from clinicians, researchers, and cancer control programs.¹ Rising incidence in younger women highlights the need for targeted interventions to improve breast symptom awareness, strengthen family history assessment, reduce diagnostic delays, and inform future screening policy discussions, even if the current data are not designed to answer those questions directly. Likewise, increasing distant-stage incidence among several Asian American, Native Hawaiian, and Pacific Islander populations raises concern that some communities may endure barriers to early diagnosis that require culturally and linguistically tailored responses,¹ alongside policies that address deep-rooted barriers to care. Previous research showing inequities in breast and cervical cancer screening among Asian American women has further supported the need to avoid treating these populations as homogeneous in outreach and prevention efforts.⁷

The Gomez et al¹ study also highlights how far data equity efforts still have to go. The updated Surveillance, Epidemiology, and End Results specialized database makes this analysis possible, and that advance should be recognized. But important limitations remain, including increasing numbers of cases categorized as not otherwise specified, uncertainty in denominator estimation, sparse subgroup-specific screening and risk factor data, and the growing complexity of classifying multiracial populations.¹ These challenges are not merely technical. They shape what questions can be asked, which populations can be seen, and how resources are allocated. More than 2 decades after federal attention turned to improving data collection and reducing disparities for Asian American, Native Hawaiian, and Pacific Islander populations, investment has still lagged. Prior work showed that only a very small proportion of National Institutes of Health clinical research funding explicitly focused on Asian American, Native Hawaiian, and Pacific Islander populations, helping explain why important evidence gaps remain.²

The path forward requires more than increased surveillance; it demands the integration of surveillance, research, and clinical action. To uncover the drivers of these trends, epidemiologic studies must capture migration histories, such as nativity and arrival cohorts, alongside premigration and postmigration social and environmental exposures.⁵ Furthermore, linking these factors to cancer subtypes and tumor genomic data will be essential to understanding how shifting life course exposures intersect with and influence tumor biology.^{1,6} Public health practitioners should prioritize culturally and linguistically tailored outreach and screening efforts designed with specific Asian American, Native Hawaiian, and Pacific Islander communities rather than generic approaches for a broad, aggregated population.^{3,4} Clinicians and health systems, as well as medical and scientific educators, should recognize that Asian American, Native Hawaiian, and Pacific Islander populations are not uniformly low risk and that subgroup-specific patterns may matter for awareness, outreach, and care delivery. Teaching and reference materials should likewise avoid overly broad, aggregated statements that obscure clinically meaningful heterogeneity across Asian American, Native Hawaiian, and Pacific Islander populations. Furthermore, health information systems should continue to improve disaggregation and harmonization, particularly through more accurate, standardized, and interoperable capture of detailed race and ethnicity data across electronic health records and cancer

registries, so that future analyses can move from identifying inequities to guiding precise intervention.²⁻⁴ Finally, advancing Asian American, Native Hawaiian, and Pacific Islander cancer equity will require investment in surveillance and research to identify inequities and evaluate interventions, along with protecting health care funding mechanisms that support implementation of research-driven strategies and ensure timely access to screening, diagnosis, and care.

Gomez et al¹ make an important contribution by showing what becomes visible when Asian American, Native Hawaiian, and Pacific Islander populations are examined with greater specificity. The core lesson of this study is not only that breast cancer incidence is rising but also that aggregation can hide urgency and that without data equity, cancer equity will remain out of reach.²⁻⁴

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